



Specimen Comments

SPECIMEN

1. Left distal resection margins
2. Right distal resection margins
3. Bladder and prostate

CLINICAL DETAILS

Invasive bladder cancer

MACROSCOPY

1. Tubular piece of tissue measuring 12 x 5mm diameter. No focal lesion.
2. Tubular piece of tissue measuring 14 x 6mm diameter.
3. Bladder and prostate with attached fibrofatty tissue. The bladder measures 40mm SI x 35mm ML . The prostate measures 30mm SI, 30mm ML x 21mm AP.

Attached

fibrofatty tissue measures up to 120mm maximum dimension. No lymph nodes identified.

In the left lateral wall of the bladder there is an ulcerated area 20mm.

Underlying this is a hard tumour extending into perivessicular fat, measuring 25mm SI by approximately 25mm ML. The tumour extends for a depth of 22mm and is

very close to the left lateral soft tissue circumferential resection margin.

Elsewhere, on the right side of the urethral orifice is an area of mucosal irregularity 15 x 12mm.

MICROSCOPY

The tumour seen in the left lateral wall of the bladder is a sarcomatoid variant of transistional cell carcinoma/high grade urothelial carcinoma. The tumour is composed predominantly of high grade malignant spindle cells with no heterologous elements evident. Superficially, the morphology of the tumour is more reminiscent of a solid conventional high grade urothelial carcinoma and there is some overlying carcinoma in situ from which the tumour appears to be arising. There is no definite evidence of lymphovascular invasion.

The tumour invades through the muscularis propria into the perivessicular fat (pT3) and is 2mm from the circumferential resection margin at its closest point (block 3B).

The macroscopically abnormal area near the bladder neck shows evidence of oedema and increased vascularity associated with urothelial hyperplasia. There is some patchy dysplasia in this area which just amounts to CIS in a few foci. The prostatic urethra shows evidence of similar oedema and increased vascularity associated with von Brunn's nests and hyperplasia but there is no evidence of dysplasia and the prostatic resection margin is clear of neoplasia.

The prostate, vas deferens and seminal vesicles show no significant abnormality.

The left and right distal ureteric resection margins (specimens 1 and 2) show no definite evidence of carcinoma in situ.

SUMMARY

Cystoprostatectomy:

Sarcomatoid variant of high grade urothelial carcinoma.(G3,pT3)

2mm clearance from left lateral circumferential resection margin.

Adjacent carcinoma-in-situ.

*Carcinoma, urothelial TBS 8/20/3*  
*Carcinoma, transitional cell, sarcomatoid*  
*Site Bladder, lateral wall*  
*8/22/3*  
*C672*  
*JWH/30/14*

Urothelial resection margins clear.

Ref:

Pathologists -

|                                |            |     |              |
|--------------------------------|------------|-----|--------------|
| Criteria                       | iw 11/9/14 | Yes | No           |
| Diagnosis Discrepancy          |            |     | ✓            |
| Primary Tumor Site Discrepancy |            |     | ✓            |
| HIPAA Discrepancy              |            |     | ✓            |
| Prior Malignancy History       |            |     | ✓            |
| Dual/Synchronous Primary Noted |            |     | ✓            |
| Case is (circle):              | QUALIFIED  |     | DISQUALIFIED |
| Case reviewed by:              | 11/9/14    |     |              |